

SENATE No. 1578

The Commonwealth of Massachusetts

PRESENTED BY:

Liz Miranda

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act measuring the impact of Long COVID in the Commonwealth.

PETITION OF:

NAME:

Liz Miranda

DISTRICT/ADDRESS:

Second Suffolk

SENATE No. 1578

By Ms. Miranda, a petition (accompanied by bill, Senate, No. 1578) of Liz Miranda for legislation to create an evidence-based culturally-specific patient navigation pilot program for people living with a Long COVID diagnosis. Public Health.

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act measuring the impact of Long COVID in the Commonwealth.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Chapter 111 of the General Laws, as so appearing in the 2022 Official
2 Edition, is hereby amended by inserting after section 24O the following section:-

3 Section 24P. (a) The department, subject to appropriation, shall develop and implement
4 an evidence-based culturally-specific patient navigation pilot program for people living with a
5 Long COVID diagnosis, aligned with the National Academies of Sciences, Engineering, and
6 Medicine (NASEM) Long COVID definition.

7 (b) The department shall design the pilot program to: (i) reduce barriers for Long COVID
8 patients in accessing timely and knowledgeable medical treatment, (ii) provide referrals for
9 clinical care and nonclinical support to patients with Long COVID and their families, (iii)
10 facilitate access to medical information and resources to help meet daily needs and emotional
11 support, and (iv) integrate within healthcare teams for maximum efficacy.

(c) The department shall develop guidelines to standardize Long COVID patient navigator care, which could include appropriate training to ensure skills and knowledge needed to deliver the proposed service.

(d) The pilot program shall include, but not be limited to, the following activities:

(i) collecting and sharing data;

(ii) making initial contact with patients, conducting social and medical needs assessments;

(iii) providing navigation on clinical needs including assistance for patients with Long COVID to identify appropriate subspecialty care, making referrals and connections to clinicians, facilitating communication among providers and with the patient's health care team, scheduling medical appointments, and optimizing insurance coverage and disability insurance coverage;

(iv) providing navigation on social and emotional support needs including providing connections for patients with Long COVID to lay support group services in order to address stress for patients with long COVID and to social services in order to address issues that include transportation, language barriers and interpretation, food insecurity, housing insecurity, unemployment assistance, connections to community-based organizations and other services;

(v) providing resources to inform shared decision-making and the availability of social and community services;

(vi) establishing reimbursement for patient navigator services and supervision of such services

32 (e) Not later than one year after the implementation of the pilot program said department
33 shall provide a report to the clerks of the house and the senate and to the joint committees on
34 public health and health care financing. The report shall include, but not be limited to; (i) the
35 activities of the pilot, including services provided, the geographical locations of services, (ii) the
36 number of patients who participated and the region where they live, and (iii) the advisability of
37 expanding the pilot program.